

Encounter Date: [REDACTED]

UID: DE5B5338-18C3-44AA-AB8E-ADFC18EA5C87
TCGA-GV-A3QF-01A-PR

Redacted



SURGICAL PATHOLOGY (Order: [REDACTED])

Patient Name

Sex

DOB

Results

Specimen #: [REDACTED]
Submitting Physician:

ICP-0-3
carcinoma, urothelial, NOS
8/20/13
Site: bladder, anterior wall
2.29.12 RO C67.3

FINAL DIAGNOSIS

1. Right distal ureter, biopsy (A) - Benign ureter, negative for neoplasm.
2. Left distal ureter, biopsy (B) - Benign ureter, negative for neoplasm.
3. Urinary bladder, radical cystoprostatectomy (C) - Invasive high-grade papillary urothelial carcinoma, 4.2 cm.
 - The tumor invades through muscularis mucosa (lamina propria) and deeply involves the perivesical fat.
 - Extensive lymphovascular invasion present.
 - Bilateral ureteral, urethral and soft tissue resection margins are negative for tumor.
 - Two perivesical lymph nodes, positive for metastatic urothelial adenocarcinoma (2/2).
4. Prostate, radical cystoprostatectomy (C) - Adenocarcinoma of the prostate, Gleason score 3+4=7, involving less than 5% of the total prostatic tissue.
 - The tumor is confined within the prostate.
 - Bilateral seminal vesicles and prostatic resection margins are negative for tumor.
5. Left pelvic lymph nodes, excision (D) - Seven lymph nodes, negative for malignancy (0/7).
6. Left common iliac lymph nodes, excision (E) - Four lymph nodes, negative for malignancy (0/4).
7. Left paraaortic lymph nodes, excision (F) - Three lymph nodes, positive for metastatic urothelial carcinoma (3/3).
8. Right pelvic lymph nodes, excision (G) - Eleven lymph nodes, negative for malignancy (0/11).
9. Presacral lymph nodes, excision (H) - Three lymph nodes, positive for metastatic urothelial carcinoma (3/6).
10. Right common iliac lymph nodes, excision (I) - Four lymph nodes, positive for metastatic urothelial carcinoma (4/4).

*** COMMENT

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIFAA Discrepancy		
For Malignancy History		
Qual/Synchronous Primary Listed		
Case is (for):		
Reviewer Initials		

Prostate
2.20.12
12/20/12

3. Specimen Type: Radical cystoprostatectomy

Tumor Size, Greatest dimension: 4.2 cm

Histologic Type: Urothelial carcinoma

Urothelial Carcinoma: (WHO/ , High-grade

Adenocarcinoma and Squamous Carcinoma: N/A

Associated Epithelial Lesions: Flat urothelial carcinoma in situ

Margins:

Margins uninvolved by invasive carcinoma

Venous/Lymphatic (Large/Small Vessel) Invasion (V/L): Present

Direct Extension of Invasive Tumor: Perivesical fat

Pathologic Stage:

Primary Tumor: (pT)

pT3b: Macroscopically invades perivesical fat (extravesical mass)

Regional Lymph Nodes: (pN)

pN2: Metastasis in multiple lymph nodes (true pelvis)

Specify: Number examined: 37; Number involved (any size): 12

Distant Metastasis (pM): pMX: Cannot be assessed

4. Gleason score: Primary 3, Secondary 4, Total 7

% of 4 and/or 5: 1-10%

% of 3: 91-100%

Androgen deprivation effect:

Negative

Apical involvement:

Negative

Apical margin involvement:

Negative

Extraprostatic extension (EPE): Negative

Margin: Negative

Bladder neck shave: Negative

Areas of involvement: left peripheral zone

Principal area of involvement: left peripheral zone

Focality: unifocal

Volume Low

Greatest dimension of larger tumor nodule: 2 mm

Ejaculatory duct apparatus: Negative

Seminal vesicles involvement (SVI): Negative

Peri-SV involvement: Negative

Vas deferens: N/A

Peri-vas involvement: N/A

High-grade PIN: Present

Neuroendocrine differentiation: Negative

Lymph nodes: See bladder template

Diameter of largest lymph node metastasis (mm) N/A

Lymphovascular invasion: Negative

Other diagnoses: Atrophy and inflammation

Pathologic Stage: pT2a, pN0, pMx

(Electronic Signature)

SPECIMEN SUBMITTED

A: RIGHT DISTAL URETER
B: LEFT DISTAL URETER
C: BLADDER AND PROSTATE
D: LEFT PELVIC LYMPH NODES
E: LEFT COMMON ILIAC LYMPH NODES
F: LEFT PARA AORTIC LYMPH NODES
G: RIGHT PELVIC LYMPH NODES
H: PRE-SACRAL LYMPH NODES
I: RIGHT COMMON ILIAC LYMPH NODES

CLINICAL DATA
BLADDER CANCER

INTRAOPERATIVE CONSULT DIAGNOSIS

A. Negative for neoplasm (Dr. .
B. Negative for neoplasm (Dr.

GROSS DESCRIPTION

A. Received fresh at the frozen desk labeled "right distal ureter" is a specimen consisting of red-tan soft tissue with a central lumen measuring 1.5 x 0.8 x 0.5 cm. The specimen is totally submitted for frozen as FSA1.

B. Received fresh at the frozen desk labeled "left distal ureter" is a specimen consisting of one fragment of red-tan soft tissue with a central lumen measuring 1.2 x 0.8 x 0.5 cm. The specimen is totally submitted for frozen as FSB1.

[REDACTED]

C. Received in formalin labeled "bladder and prostate" is a specimen consisting of a bladder and prostate measuring 17 x 10 x 5 cm. The specimen has been previously opened anteriorly. The bladder is inked black and the prostate is inked yellow on the left and blue on the right. The prostate measures 6 cm transversely, 4.5 cm anterior posteriorly, and 5 cm craniocaudally. The seminal vesicles are unremarkable. The prostate shows no masses or lesions. The bladder reveals a large fungating mass in the left anterior wall measuring 2.3 x 1.2 x 0.8 cm. Inferior to this mass stretching from the anterior to the left wall is an ulcerated area measuring 1.5 x 0.7 cm. The area of ulceration abuts the left ureter orifice. Additionally seen is a focal area of hemorrhage in the posterior wall measuring 0.4 x 0.2 cm. Sectioning reveals the area under the ulcer and fungating mass to contain an infiltrative firm white lesion which extends to 0.5 cm from the inked black soft tissue margin. The remainder of the bladder mucosa is unremarkable. The prostate shows no masses or lesions. Representative sections are submitted as follows: C1 distal right ureter, C2 distal left ureter, C3 urethral margin, C4 left apex, C5 right apex, C6 3 mm left anterior, C7 3 mm right anterior, C8 3 mm left posterior, C9 3 mm right posterior, C10 6 mm left anterior, C11 6 mm right

[REDACTED]
anterior, C12 6 mm left posterior, C13 6 mm right posterior, C14 mid left anterior, C15 mid left middle (inked anterior), C16 mid left posterior, C17 mid right anterior, C18 mid right middle (inked anterior), C19 mid right posterior, C20 base left anterior, C21 base left lateral posterior, C22 base left mid medial posterior, C23 base right anterior, C24 base right posterior, C25 left seminal vesicle, C26 right seminal vesicle, C27 ulcer (submitted inferiorly to superiorly), C28-C29 deepest extent of white mass under ulcer, C30 ulcer with fungating mass, C31-C32 ulcer with mass, C33 remainder of mass, C34 remainder of mass (totally submitted), C35 superior bladder wall, C36 urethra, C37 posterior bladder wall with hemorrhage, C38 right bladder wall, C39 right ureter representative section, C40 left ureter with area of ulceration, C41-C42 possible lymph nodes.

[REDACTED]

D. The specimen is received fresh at the surgical desk labeled "left pelvic lymph nodes" and consists of a 6.5 x 6.0 x 1.0 cm segment of fibroadipose tissue. Sectioning reveals seven nodules consistent with lymph nodes. These nodules are submitted in cassettes D1-D3.

E. The specimen is received fresh at the surgical desk labeled "left common iliac lymph nodes" and consists of 3.2 x 2.0 x 1.0 cm segment of fibroadipose tissue. Sectioning reveals four nodules consistent with lymph nodes. These nodules are submitted in cassettes E1-E4.

F. The specimen is received fresh at the surgical desk labeled "left para-aortic lymph nodes" and consists of two nodules surrounded by fibroadipose tissue aggregating to 6.0 x 2.7 x 2.2 cm. Sectioning reveals a solid mass with red-tan parenchyma with areas of granular appearing parenchyma which is more of a yellow tan color. Sections are submitted as follows: F1 smaller nodule, bisected, F2-F3 representative sections of the larger nodule.

G. The specimen is received fresh at the surgical desk labeled "right pelvic lymph nodes" and consists of an 8.0 x 5.0 x 2.0 cm segment of fibroadipose tissue with multiple nodules contained within. These nodules are submitted as follows: G1-G2 multiple nodules, G3 single nodule sectioned, G4 five nodules, G5 three nodules.

H. The specimen is received fresh at the surgical desk labeled "presacral lymph nodes" and consists of a 4.0 x 2.0 x 1.0 cm segment of fibroadipose tissue with nodules consistent with lymph nodes contained within. The specimen is submitted as follows: H1-H2 fibroadipose tissue with multiple nodules, H3 single nodule, sectioned.

I. The specimen is received fresh at the surgical desk labeled "right common iliac lymph node" and consists of a 6.0 x 4.0 x 1.0 cm segment of fibroadipose tissue with multiple nodules consistent with lymph nodes contained within. Sections are submitted as follows: I1-I6 nodules, one per cassette.

[REDACTED]

Patient ID #: [REDACTED]
DOB: [REDACTED] (Age: [REDACTED])
Date of Report: [REDACTED]
Date of Procedure: [REDACTED]
Date of Receipt: [REDACTED]

[REDACTED]

Submitted by: [REDACTED]

Location: [REDACTED]

Test performed by: [REDACTED]

Lab and Collection

SURGICAL PATHOLOGY (Order # [REDACTED] on [REDACTED] - Lab and Collection Information

Result History

SURGICAL PATHOLOGY (Order # [REDACTED] on [REDACTED] - Order Result History Report.

Result Information

Result Date and Time	Status	Provider Status
[REDACTED]	Final result	Reviewed

MyChart Status:

This result is currently not released to

[Display Full Result Report](#)

[Display Order Report](#)